

**العمر(تاريخ الميلاد) : 15-12-1970 المهنة : ربه منزل**

التوقيع: (-----)

**التعليق على التشخيص : -----**

رئيس  
المجلس الطبي المتخصص

|  |
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|  |
|--|

|                                                                         |                                                                                     |
|-------------------------------------------------------------------------|-------------------------------------------------------------------------------------|
| <b>Name:</b> حنان شعبان حسن عثمان<br><b>National ID:</b> 27012152602162 |  |
|-------------------------------------------------------------------------|-------------------------------------------------------------------------------------|

|           |
|-----------|
| تقرير طبي |
|-----------|

|                  |
|------------------|
| السيد            |
| تحية طبية وبعد,, |

|                       |                       |
|-----------------------|-----------------------|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم:                 | رقم طبي:              |

|                                          |
|------------------------------------------|
| نتشرف بأن نوضح فيما يلي تقريراً عن حالته |
|------------------------------------------|

|                                             |                                             |
|---------------------------------------------|---------------------------------------------|
| تاريخ وساعة الدخول :<br>-- --:-- yyyy-mm-dd | تاريخ وساعة الخروج :<br>PM 01:17 05-07-2026 |
|---------------------------------------------|---------------------------------------------|

|                                        |
|----------------------------------------|
| وبعد الكشف والفحوصات والمتابعة وجد أنه |
|----------------------------------------|

|                                                                                                  |
|--------------------------------------------------------------------------------------------------|
| المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT Chest & abdomen and pelvis |
|--------------------------------------------------------------------------------------------------|

|                         |                                   |
|-------------------------|-----------------------------------|
| التاريخ :<br>05-07-2026 | الوقت :<br>-- --:--               |
|                         | الطبيب المعالج :<br>د/شيماء لاشين |



٢٠١٩/٠٤

٢٧.١٢١٥٢١.٢١٦٢

رؤية متوزن

متوزجة

مسلمة

عشرى المراهق حسن

البلقة سارية حق ٢٠٢١/٠٣/٢٧



HY8598805

مملكة فلسطين الشعبية

حلق

شعبان حسن عطين

التحرير

مركز بير - البيرة

٢٧٠١٢١٥٢١٠٢١٦٢

MDT FORM

31 / 12 / 2025 G

|                   |          |                |               |
|-------------------|----------|----------------|---------------|
| Name              | حنا حبان | MRN            | 3027 / 2021   |
|                   |          | ID             | CV.191057.917 |
| Age               | 55       | Phone          | 01126777135   |
| Menopausal status | Hot      | Marital status |               |
| NRS SCORE         |          | Family history |               |
| BMI               |          | REASON OF MDT  | AI Shift      |
| CO morbidity      |          |                |               |

Clinical Data

|     |     |        |       |   |   |
|-----|-----|--------|-------|---|---|
| BX: | IDC |        |       | R | L |
| ER: | PR: | HER 2: | Ki67: |   |   |
| 7/8 | +   | +      | 3     |   |   |

Sonomammography:

2022: Rt breast + Ca

Mets Workup:

ER 7/8, Her2 + 3

NACT TCHP → Rt BCS & PCR

adjuvant Hercept 2022

Recommendations:

& Na on hormonal th  
Tamoxifen Since then "2022

11 / 2025 → FSH 23, E2 14 "PostMen  
for Shift to AI for 35y

MDT OUTCOME & PLAN

f. fenza "complete Zylall"  
3-4y + Tamoxifen

SIGNATURE

|                  |         |           |           |           |             |
|------------------|---------|-----------|-----------|-----------|-------------|
| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY | Nutrition | Psychiatric |
|                  |         |           |           |           |             |

|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |                       |                                                                                     |
|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------|-------------------------------------------------------------------------------------|
| Name: <div>حسان شعبان حسن</div>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |                       |  |
| MR. : 5082                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |                       |                                                                                     |
| Detailed Medical Report                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |                       |                                                                                     |
|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |                       | السيد                                                                               |
| تحتية طبية وبعد,,                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |                       |                                                                                     |
|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | بشأن تقرير طبي للمريض | إيماء للطلب المقدم من                                                               |
|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | رقم طبي:              | بقسم:                                                                               |
| ننشرف بأن نوضح فيما يلي تقريراً عن حالته                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |                       |                                                                                     |
|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | تاريخ وساعة الخروج :  | تاريخ وساعة الدخول :                                                                |
| <p>وبعد الكشف والفحوصات والمتابعة وجد انه</p> <p>The patient has a history of right breast cancer treated with neoadjuvant chemotherapy (NACT) followed by right breast conservation surgery (Rt CBS) with a ypT0N0 pathological result in June 2022. Post-operative management included radiation therapy (PORT) and adjuvant endocrine therapy with tamoxifen. The most recent breast-specific monitoring was a breast-specific mammogram (BSM) performed in September 2025, which was free of disease, and a bone scan conducted in July 2025, also showing no metastatic activity.</p> |                       |                                                                                     |
| تحريرا في ,                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                |                       |                                                                                     |
|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | التاريخ :             | 20-09-2026                                                                          |
|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | الوقت :               |                                                                                     |
|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | ندى حسن صلاح كامل     | الطبيب المعالج :                                                                    |
| يعتمد ,                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |                       |                                                                                     |

|                                                |                                                     |
|------------------------------------------------|-----------------------------------------------------|
| Patient Name : الأستاذة / حنان شعبان حسن عثمان | Request No. : 139681                                |
| Patient No. : 27012152602162                   | Request Date : 23/08/2021                           |
| Path No. : SR-392                              | Verification Date : 30/08/2021                      |
| Sex : Female                                   | Financial Category : نفقة الدولة - أرقام دار السلام |
| Age : 50 Y                                     | Department : Out Patient                            |
| Referring Doc :                                |                                                     |

## Pathology Report

### Clinical Data:

Rt breast cancer & Rt axillary lymphadenopathy (FNA done)

### Nature of the specimen:

Slide revision

### Gross Examination:

one block labeled as (5040/21), and 3 slides.

one block labeled as (982/21), and broken smear slides

### Microscopic examination:

Sections examined reveals an infiltrating malignant epithelial growth. The tumour is made up of groups and small sheets of moderately pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate.

cytologic examination of the smear slide and slide of cell block revealed positivity for malignancy.

### Diagnosis:

Rt breast mass & Rt axillary LN (FNA), slide revision:

invasive duct carcinoma GII.

cytology and cell block positive for malignancy.

#### • Biomarkers:

o ER: strong nuclear reaction in 60% of tumor cells (Allred score 7/8)

o PR: moderate nuclear reaction in 10% of tumor cells (Allred score 4/8)

o Her2: strong complete membrane reaction in > 10% of tumor cells (score 3+)

Prof. Amal Hareedy  
Dr. Rana Fathy



|                                           |                   |
|-------------------------------------------|-------------------|
| Patient Name: Hanan Shaaban Hassan Othman | Date : 06/02/2024 |
| Exam : MAMMOGRAPHY BOTH                   | Age : 53 Year(s)  |
| Medical ID: 327/2021                      | Pat. ID: 22571    |

**BILATERAL BREAST MAMMOGRAPHY AND HIGH  
RESOLUTION ULTRASOUND**

**Indication:**

Right breast conserving therapy

**Technique**

- Bilateral MLO and CC views
- Bilateral breast ultrasound

**Report:**

- Scattered fibro glandular density (ACR B)

**Right breast:**

- Operative bed parenchymal distortion and scarring with no sizable mass
- No suspicious mass lesions, micro calcifications
- No sizable collection
- Normal skin thickness.
- No regional lymphadenopathy

**Left breast:**

- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

**Opinion:**

- Post right BCS status showing no evidence of recurrence (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Best regards,  
Dr. Eman Abdel Razik, MSc, FEBR  
Dr. Amira H Radwan MD,  
Breast Imaging consultant,  
NCH, Cairo University

Handwritten signature and date: 6/2/2024



|                                           |                   |
|-------------------------------------------|-------------------|
| Patient Name: Hanan Shaaban Hassan Othman | Date : 03/11/2022 |
| Exam : ultrasound BREAST (BOTH SIDE )     | Age : 52 Year(s)  |
| Medical ID: 3027/2021                     | Pat. ID: 22571    |

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION  
ULTRASOUND**

**Indication:**

Patient had right CBS in 6/2022

**Technique**

- Bilateral MLO and CC views
- Bilateral breast ultrasound

**Report:**

- The breasts are heterogeneously dense which may obscure small lesions (ACR C)
- right breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific axillary LN, bilaterally with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

**Opinion:**

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Benign findings of the left breast (BIRADS 1)

Yours truly

Signature Dr. Engy Abdallah

03/11/2022 2:40 PM



|                   |                                           |
|-------------------|-------------------------------------------|
| Date : 06/02/2024 | Patient Name: Hanan Shaaban Hassan Othman |
| Age : 53 Year(s)  | Exam : ultrasound ABDOMEN& PELVIS         |
|                   | Medical ID: 3027/2021                     |

### **ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:**

#### **Liver: -**

- Average in size with homogenous echopattern
- No intra hepatic biliary radicle dilatation or focal lesions.
- Portal vein is patent

#### **Gall Bladder: -**

- Average distention with normal wall thickness with no stones.
- CBD is not dilated.

#### **Spleen: -**

- Average in size displaying homogenous echopattern, no focal lesions.

#### **Kidneys: -**

- Are of normal size , well differentiated & no gross stones or back pressure changes or space occupying lesion could be seen.

**Para-aortic and pancreatic area: are concealed by gases.**

**No ascites or free fluid or collection seen .**

#### **Urinary Bladder: -**

- Average filling with no stones or focal lesion seen

### **Opinion:**

- Unremarkable study

Best regards  
Dr/Shahd M. Fouad ,Msc

|                                          |                                       |
|------------------------------------------|---------------------------------------|
| PATIENT NAME: HANAN SHABAN HASSAN AOSMAN | PATIENT ID #: 2021159864              |
| GENDER: Female                           | ORDER #: 1146113,1146119              |
| AGE: 50 y                                | DATE OF SERVICE: 25-Jul-2021 11:20 AM |

Report: - all studies

**MULTI-SLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS  
WITH TRIPHASIC STUDY OF THE LIVER REVEALED:**

**(I) Chest:**

- Right breast heterogeneously enhancing mass is seen measuring 2.6 cm.
- Prominent right axillary lymph nodes, the largest measuring 1.9X0.8 cm.
- Patent central air ways with well inflated both lungs.
- Both lung fields are clear showing no parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

**(II) Abdomen and pelvis:**

- The liver is average in size showing homogenous CT density and enhancement with no focal lesions.
- Patent and homogeneously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- Bulky uterus.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- Dorsolumbar spondylosis.

**OPINION:**

- **No CT evidence of metastatic pulmonary or hepatic metastatic disease.**

Reported By,

Dr. Amr Farouk ,MD  
Dr. Mahmoud Saleh, MD

25-Jul-2021 02:52 PM



EAJMD36749567

وہابی فائدہ

وزارة الصحة والسكان  
أمانة المراكز الطبية المتخصصة  
**مستشفى أورام دار السلام**  
هرمل ، سابقا  
إدارة الجودة

## تذكرة العيادة الخارجية

التاريخ: ١٦ / ٢ / ١٤٣٨ هـ رقم القيد: ٥٠٩٧ الرقم القومي: ٩٦٩٦٩٦٩٦  
اسم المريض: محمد بن عبد الله بن محمد بن علي النور  
الوظيفة: مدير عام شركة  
العنوان: الرياض - حي النور  
الهاتفون:  
مجموعة: أ ○ ب ○ ج ○  
توقيع كاتب الاذن:

### General Examination

**vital signs:**

| ضغط الدم      | درجة الحرارة | النبض | الأنفول | الوزن |
|---------------|--------------|-------|---------|-------|
|               |              |       |         |       |
| توقيع التمريض |              |       |         |       |

Marital History: ☐ Married ☐ Single ☐ Widow ☐ Divorced  
 Menopausal Status: ☐ Menopausal ☐ Premenopausal ☐ Postmenopausal  
 Children: ☐ Yes ☐ No Number: .....  
 Allergy: ☐ Yes ☐ No Comment: .....  
 Adverse drug reaction: ☐ Yes ☐ No Comment: .....  
 Smoking: ☐ Yes ☐ No Comment: .....  
 Pain: ☐ Yes ☐ No Comment: .....

**Complaints:** \_\_\_\_\_

Present History: .....

Past History (hospital admission & surgery):

**Family History:** .....



|                                           |                   |
|-------------------------------------------|-------------------|
| Patient Name: Hanan Shaaban Hassan Othman | Date : 06/02/2024 |
| Exam : MAMMOGRAPHY BOTH                   | Age : 53 Year(s)  |
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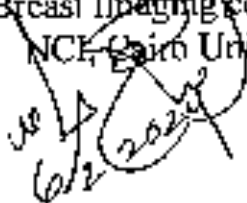
**Left breast:**

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Best regards,  
Dr. Eman Abdel Rāzīk, MSc, FEBR  
Dr. Amira H Radwan MD,  
Breast Imaging consultant,  
NCE, Cairo University

  
06/2/2024



|                   |                                           |
|-------------------|-------------------------------------------|
| Date : 06/02/2024 | Patient Name: Hanan Shaaban Hassan Othman |
| Age : 53 Year(s)  | Exam : ultrasound ABDOMEN& PELVIS         |
|                   | Medical ID: 3027/2021                     |

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#### **Urinary Bladder:-**

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### **Opinion:**

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Best regards  
Dr/Shahd M. Fouad ,Msc

|                                          |                                       |
|------------------------------------------|---------------------------------------|
| PATIENT NAME: HANAN SHABAN HASSAN AOSMAN | PATIENT ID #: 2021158884              |
| GENDER: Female                           | ORDER #: 1146113,1146119              |
| AGE: 50 y                                | DATE OF SERVICE: 25-Jul-2021 11:20 AM |

Report: - all studies

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**OPINION:**

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Reported By,

Dr. Amr Farouk, MD  
Dr. Mahmoud Saleh, MD

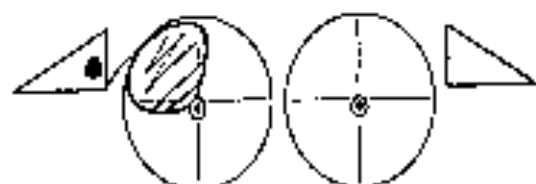
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مبادرة رئيس الجمهورية  
لدعم صحة المرأة المصرية

السلطة المصرية  
للصحة  
محطة

٢٠٢١/٨/٢٤ MDT FORM

|              |                 |
|--------------|-----------------|
| Name         | حنان تيماني حسن |
| Age          |                 |
| MRN          | ٢٧-١٥١٥٢٦-٩١٦٩  |
| ID           | 302712021       |
| Co morbidity | ١٠١٦٥٠٤٤٠٥٥     |



Clinical Data

|       |          |              |                                                           |
|-------|----------|--------------|-----------------------------------------------------------|
| Bx:   | IDC G-II | SM:          | R+ UOQ multifocal<br>suspicious enhancing mass<br>postmen |
| IHC:  |          | Mets Workup: |                                                           |
| ER:   | 718      |              | Free                                                      |
| PR:   | 418      |              |                                                           |
| Hert: | 3+       |              |                                                           |
| KITV: |          |              |                                                           |

cT<sub>2</sub> N<sub>1</sub> M<sub>0</sub> cT

COMMENT:

- Bone scan
- Echo, labs
- Clip insertion

MDT OUTCOME & PLAN

|                                                  |
|--------------------------------------------------|
| - Brain MRI                                      |
| - Neoadj Chemo AC x4 → Taxol/Hercophor           |
| + Perjeta x4 → Consider ACS if<br>responded well |

| SIGNATURE        |         |           |           |
|------------------|---------|-----------|-----------|
| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY |
|                  |         |           |           |

14-2-2022

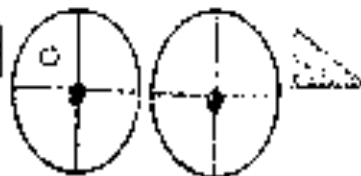


مبادرة رئيس الجمهورية  
لدعم صحة المرأة المصرية

السلطة المصرية  
للصحة  
محكمة

### MDT FORM

|              |                      |
|--------------|----------------------|
| Name         | هناء محمد            |
| Age          | 31                   |
| MRN          | 00000000000000000000 |
| ID           | 00000000000000000000 |
| Co morbidity | C.01 / 2.05          |



### Clinical Data

|                                                                                                                    |              |
|--------------------------------------------------------------------------------------------------------------------|--------------|
| Bx:                                                                                                                | SM:          |
| IHC:<br>ER: 7/8<br>PR: 6/8<br>Her2: +++<br>Ki67:                                                                   | Mets Workup: |
| cT3 N1 → yT1 N0 M0                                                                                                 |              |
| COMMENT:<br>resect and LAC → 12 who Tumor<br>+ 4 x the cystic + Pseudo<br>for CESM to assess R Conservation<br>GCS |              |

### MDT OUTCOME & PLAN

|                                                                        |
|------------------------------------------------------------------------|
| for <del>BCS</del> BCS + SLN (≥ 4LN)<br>After wire localisation<br>GCS |
|------------------------------------------------------------------------|

| SIGNATURE        |         |           |           |
|------------------|---------|-----------|-----------|
| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY |
|                  |         |           |           |



مبادرة رئيس الجمهورية  
لدعم صحة المرأة المصرية

الصحة المصرية  
صحة  
مصر

MDT FORM

2022/3/2

|                   |           |                |             |
|-------------------|-----------|----------------|-------------|
| Name              | جاءة محمد | MRN            | ٢٠٥٧٥٠٢١    |
| Age               | 54        | ID             | ٥٧٠٤١٥٧٠٤١٧ |
| Menopausal status |           | Phone          | ١٢٦٧٧٠٤٤٧٥  |
| OCP               |           | Marital status |             |
| Co morbidity      |           | Family history |             |

### Clinical Data

Rx: TAC 2 II

ER: 7/9 PR: 4/8 HER 2: ++ Ki67:

Sonomammography:

Mets Workup:

Free

CT INI

→ yT.NoM. (PCR)

Recommendations:

Acad to Rt Breast. post BC+I Live

post RACT pathology: → pathological complete response SLNB → o/s pTo pNo

for post operation re MDT

MDT OUTCOME & PLAN

- Adj Trastuzumab SC. x 13 doses
- PORT to Rt. Breast & SC
- Adj hormonal th Arimidex x 54

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY



Med. Record No.: 2021158354  
Patient Name: HANAN SHABAN HASSAN AQSMAN  
Sex: Female DOB: 12/15/1970  
Nationality: EGYPTIAN  
Unit: External Clinic

**SURGICAL PATHOLOGY - Final Anatomical Diagnosis**  
**Biopsy Small - (1 Site)**

|                 |                     |                   |                         |
|-----------------|---------------------|-------------------|-------------------------|
| Specimen Id     | 2107060250          | Sample Type       | Tissue                  |
| Accession No    | SUR21-S040          | Requesting Doctor | Abd ALLAH Mohamed Gamil |
| Collection Date | 7/6/2021 12:43:04PM |                   |                         |
| Receive Date    | 7/6/2021 12:43:12PM |                   |                         |

**Clinical Information**

US guided biopsy from RL breast UOQ suspicious lesion (BIRAD 4C)

**Gross Picture**

Multiple cores range from 4 up to 8 mm. Totally embedded in one block

**Microscopic Picture**

A case of invasive duct carcinoma, NST, grade II. There is associated intermediate grade DCIS (solid) pattern, constituting 7% of tumor focus. Stroma shows moderate inflammatory response. Lymphovascular emboli are seen.

**Diagnostic Summary**

**Tumour type:**

Invasive carcinoma, NST, grade II.

**Laterality**

Right

**Location**

Upper outer quadrant

**Specimen type:**

Core biopsy.

**Histological grade**

**Mitosis score (1-3):**

2

**Tubular formation score (1-3):**

3

**Nuclear grade (1-3):**

2

**Histological grade (1-3):**

2



Med. Record No.: 2021159864  
Patient Name: HANAN SHABAN HASSAN AQSMAN  
Sex: Female DOB: 12/15/1970  
Nationality: EGYPTIAN  
Unit: External Clinic

**CYTOPATHOLOGY - Final Anatomical Diagnosis**  
**(Cytology - 1 Site) فحص خلوي (1 Site)**

Specimen Id: 2107050254 Sample Type: Fluid/ Smear  
Accession No: CYTO21-982 Requesting Doctor: Abd ALLAH Mohamed Gamil  
Collection Date: 7/6/2021 12:43:07PM  
Receive Date: 7/6/2021 12:43:18PM

**Nature of Specimen:**

LUS guided FNAC from Rt axillary lymph node.

**Gross Picture:**

Four sections were prepared and Pap stained. One cell block was prepared.

**Microscopic Picture:**

Cell block and smears show scattered groups of malignant epithelial cells. The groups show glandular differentiation. Pleomorphism is moderate. Picture is of metastatic adenocarcinoma of possible breast origin.

**Diagnosis:**

Rt axillary lymph node, LUS guided biopsy:  
Positive for metastatic adenocarcinoma, consistent with primary breast origin.

**Comments:**

For clinico-radiologic correlation.

**Category Code (LN)**

[C8: Malignant]

**Signature**

[AYA MOHAMMED ISMAIL]

**Co-Signed by:**

[GIHADA MOHAMED MD PhD]

Peritumoural lymphovascular invasion  
Absent

## Diagnosis

Rt breast UQC mass, US guided biopsy:  
Invasive duct. carcinoma, NST, grade I.

## Comments

ER, PR and HER2 were ordered

## Biopsy reporting category

BSI= Invasive cancer

## Signature

AYA MOHAMMED ISMAIL

## Co-signed by:

[GHADA MOHAMED MD PhD.]

completed on: 7/14/2021

## Referring Specialty :

Radiology

## Type of Request :

Regular

## History, Relevant Clinical Findings & Previous Inv. Results:

breast

## Previous Pathology Result:

## Procedure Done :

## Site of Procedure Done:

## Side of Procedure Done:

Right

## Nature & Number of Specimens:

## Reason for Order:

Surgical Pathology Exam

|                                          |                                       |
|------------------------------------------|---------------------------------------|
| PATIENT NAME: HANAN SHABAN HASSAN AOSMAN | PATIENT ID #: 2021159884              |
| GENDER: Female                           | ORDER #: 1131295,1131910              |
| AGE: 50 y                                | DATE OF SERVICE: 08-Jul-2021 09:16 AM |

Report: - all studies

### CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

#### INDICATION:

- Right breast suspicious mass lesion (FNAC outside revealed malignant cells).

#### TECHNIQUE:

- Contrast Enhanced Spectral Mammography CC and MLO view.
- Complementary ultrasound

#### REPORT:

- Bilateral moderate heterogenous back ground parenchymal enhancement with:
  - Right UOQ intensely enhancing mass lesion measuring 27.6x24mm (index lesion).
  - Another right UOQ two smaller enhancing mass lesions measuring 20x14.5mm and 10x6.5mm, all measuring an area of 92x30mm away from the nipple by 66.5mm (measured on CC view).
  - Right UOQ faint non mass enhancement measuring 41x32mm, collectively with the masses measuring an area of 93x95mm away from the nipple by 66mm.
  - No significant left enhancing lesions.
- Complementary Ultrasound showed right UOQ 9 o'clock three ill defined hypoechoic solid mass lesions measuring 26x25.3mm, 14.5x6.6mm and 8.3x7.6mm respectively, the nearest from the nipple at 25mm.
- Another right 10 o'clock ill defined Isoechoic lesion (corresponding to the non mass enhancement) measuring 22.5x12.2mm.
- Normal skin thickening and breast contour.
- Normal nipple areola complex.
- Right indeterminate axillary lymph nodes.

#### OPINION:

- Right breast UOQ multifocal suspicious enhancing mass lesions as described (BIRADS 4c) for histopathological correlation.

- No left pathological mass or non mass enhancement (BIRADS1).

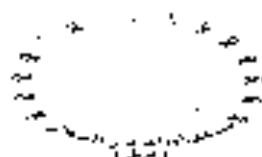
**Recommendation - all studies**

Referral to clinic

**Reported By,**

**Dr. Mai Magdy, MSc**

**06-Jul-2021 11:33 AM**



**Name : Hanan Shaban Hassan**

**Date : 30-03-2021**

### **MRI EXAMINATION OF THE BRAIN**

#### **Technique of examination:**

- Axial T1, T2 & Flair WIs "Plate 1-3".
- Sagittal T1 WIs "Plate 4".
- Post contrast axial sagittal and coronal T1 WIs "Plates 5,6,7".

#### **A known case of breast cancer.**

#### **MRI Findings:**

- Normal size shape & position of the cerebral ventricles with no midline shift or deformity.
- Normal gray–white matter interface with no areas of abnormal signal intensity.
- No evidences of mass lesions.
- No areas of abnormal enhancement are seen at the post contrast scan.
- No gross vascular abnormality.
- Normal cerebellum, brain stem & cervico – medullary junction.
- Normal sellar region.
- Normal extra axial spaces.

#### **Opinion:**

- *Normal contrast enhanced MRI study of the brain.*

Dr. Reham Anwar, MSc. FRCR

Dr m.gumaa

### Clinical Summary

Right Breast IDC Grade II

### Technique

Pre & Post contrast Mediolateral oblique and CC views  
Targeted High resolution ultrasound

### Detailed Findings

#### Left Breast:

- Dense fibroglandular parenchyma. (ACR C)
- Contrast enhanced images demonstrated
- No mass or non-mass enhancement.
- Normal skin/nipple/areolar complex.
- No pathological axillary node.

#### Right Breast:

- Dense fibroglandular parenchyma. (ACR C)
- Pre-contrast images revealed the UOQ ill-defined mass in the underlying lesions, located at 9-10 o'clock, largest measuring about 15x7mm.
- Normal skin/nipple/areolar complex.
- No definite pathological axillary lymph node seen.

### Conclusion:

- Right breast: Residual UOQ multifocal process, showing no enhancement (BIRADS 6).
- Left breast: normal study (BIRADS 1).

Best regards,

Dr. Mahi Mokhtar

Name: Hanan Shabaan Hassan

51Y

F

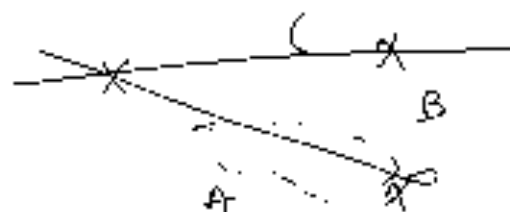
Date: 24/2/2022

### **Wire localization for right breast masses:**

- Under local anesthesia, antiseptic condition and ultrasound guidance.
- Ultrasound guided wire localization was inserted into the right breast 10 o'clock residual.
- With the following distances:
  - Whole distance of the wire till the tip: 3.2 cm
  - Perpendicular distance from skin to tip: 1.2 cm
  - Distance from perpendicular distance to skin entry: 2.9 cm.
- Pre operative breast wire localization.

**Marker was placed on the skin over the tip as well as bracketing of the distal extension of the parenchymal distortion.**

**Best Regards,  
Dr. Ahmed M. Rozeka, MSc**



A = 3.2 cm

B = 1.2 cm

C = 2.9 cm

Dr. Ahmed  
 Rozeka  
 24/2/2022



|                                           |                   |
|-------------------------------------------|-------------------|
| Patient Name: Hanan Shaaban Hassan Othman | Date : 03/11/2022 |
| Exam : ultrasound BREAST (BOTH SIDE )     | Age : 52 Year(s)  |
| Medical ID: 3927/2021                     | Pat. ID: 22571    |

## **BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND**

### **Indication:**

Patient had right CBS in 6/2022

### **Technique**

- Bilateral MLO and CC views
- Bilateral breast ultrasound

### **Report:**

- The breasts are heterogeneously dense which may obscure small lesions (ACR C)
- right breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific axillary LN, bilaterally with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

### **Opinion:**

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Benign findings of the left breast (BIRADS 1)

Yours truly

Signature Dr. Engy Abdallah

03/11/2022 2:40 PM



٨/١٠

وزارة الصحة  
المملكة العربية السعودية  
مركز العناية المتخصصة  
أمراض القلب

حناة شهاب حسن

اسم المريض ربيعي

رقم الملف الطبي

نوع التوجيه

تاريخ اعداد التوجيه

وقت اعداد التوجيه

ort

|       |     |           |         |     |                   |
|-------|-----|-----------|---------|-----|-------------------|
| EF    | 61  | 55-75%    | PW1     | 0.7 | 0.6-1.1cm         |
| LVEDD | 5.3 | 3.5-5.8cm | AOR     | 2.9 | 2-3.7cm           |
| LVEDS | 3.5 | 2-4cm     | LA      | 3.9 | 2-4cm             |
| FS    | 33  | 25-45%    | RV      | not | 0.7-2.3cm(supine) |
| SWT   | 0.7 | 0.6-1.1cm | AO.CUSP | not | 1.5-2.6cm         |

LV dimensions

LV wall thickness

LV diastolic function

SWMA

LA and Ao root dimensions

Mitral Valve

Aortic valve

Tricuspid valve

Pulmonary valve

Right side and systolic function

Intracardiac masses and effusion

Signature MSC

## Echo report

Name: *Mohammed Anwar*

Date: 24/8/2021

|       |     |           |         |     |                    |
|-------|-----|-----------|---------|-----|--------------------|
| EF    | 70  | 55-75%    | PWT     | 0.8 | 0.6-1.1cm          |
| LVEDD | 5.1 | 3.5-5.8cm | AOR     | 3.0 | 2-3.7cm            |
| LVESD | 3.1 | 2-4cm     | LA      | 3.6 | 2-4cm              |
| FS    | 35  | 25-45%    | RV      | —   | 0.7-2.3cm (supine) |
| SWT   | 0.9 | 0.6-1.1cm | AO.CUSP | —   | 1.5-2.5cm          |

LV dimensions *normal*

LV wall thickness *normal*

LV diastolic function *GI DD*

SWMA *normal*

LA and Ao root dimensions *normal*

Mitral Valve *thickened & grade II MR*

Aortic valve *normal*

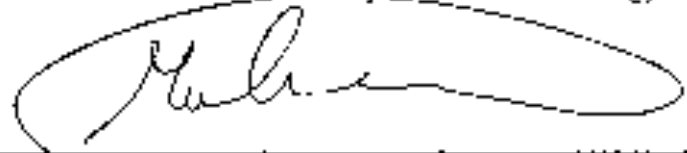
Tricuspid valve *normal & Trivial TR, RV size 19mm*

Pulmonary valve *normal*

Right side and systolic function *normal*

Intracardiac masses and effusion *normal*

Mohammed Anwar, MSc cardiology.



|             |                |          |                |
|-------------|----------------|----------|----------------|
| Name        | حنان شعبان حسن | Age, Sex | 51 Yrs, F      |
| Referred by | Prof. Dr:      | Date     | 23 / 11 / 2021 |

### Transthoracic Echocardiography Report

#### M- Mode measurements:

|           | Patient | Normal range           |                          | Patient | Normal range           |
|-----------|---------|------------------------|--------------------------|---------|------------------------|
| LVED (mm) | 52.2    | M 42 - 58<br>F 38 - 52 | Ao. Root<br>Annulus(mm)  | 24      | M 23 - 29<br>F 21 - 25 |
| LVES (mm) | 32      | M 28 - 40<br>F 27 - 35 | LA (mm)                  | 37.5    | M 30 - 40<br>F 27 - 38 |
| IVS (mm)  | 9.1     | 6 - 10                 | RV Basal<br>diameter(mm) | 32      | 25 - 41                |
| LVPW (mm) | 8.7     | 6 - 11                 | EF (%)                   | 68      | M 52 - 72<br>F 54 - 74 |

#### Comment:

- Normal sinus rhythm, with good echogenicity.
- Normal left ventricular internal dimension with normal wall thickness.
- Good left ventricular systolic function with estimated EF 68 %.
- Normal diastolic function.
- No resting segmental wall motion abnormalities.
- Normal left atrium dimension.
- Normal right ventricle structures and function.
- Mild mitral regurge.
- Normal other cardiac valves morphology and function.
- No pulmonary hypertension.
- No pericardial effusion.
- No apparent intracardiac masses or thrombi.
- Intact interatrial and interventricular septae.

#### Conclusion:

- Mild MR

Dr. Yaser Gamal  
Cardiology specialist

|             |                |          |              |
|-------------|----------------|----------|--------------|
| Name        | حنان شعبان حسن | Age, Sex | 51 Yrs, F    |
| Referred by | Prof. Dr:      | Date     | 22 / 2 /2022 |

### **Transthoracic Echocardiography Report**

#### **M- Mode measurements:**

|           | Patient | Normal range           |                          | Patient | Normal range           |
|-----------|---------|------------------------|--------------------------|---------|------------------------|
| LVED (mm) | 49.5    | M 42 – 58<br>F 38 – 52 | An.Ront<br>Annulus(mm)   | 24      | M 23 – 29<br>F 21 – 25 |
| LVES (mm) | 31.4    | M 25 – 40<br>F 22 – 35 | LA (mm)                  | 36.8    | M 30 – 40<br>F 27 – 38 |
| IVS (mm)  | 8.3     | 6 – 10                 | RV Basal<br>diameter(mm) | 32      | 25 – 41                |
| LVPW (mm) | 8.3     | 6 – 10                 | EF (%)                   | 66      | M 52 – 72<br>F 54 – 74 |

#### **Comment:**

- Normal sinus rhythm, with good echogenicity.
- Normal left ventricular internal dimension with normal wall thickness.
- Good left ventricular systolic function with estimated EF 66 %.
- Normal diastolic function.
- No resting segmental wall motion abnormalities.
- Normal left atrium dimension.
- Normal right ventricle structures and function.
- Mild mitral regurge.
- Normal other cardiac valves morphology and function.
- No pulmonary hypertension.
- No pericardial effusion.
- No apparent intracardiac masses or thrombi.
- Intact interatrial and interventricular septae.

#### **Conclusion:**

- Mild MR.

*Dr. Yaser Gamal*  
*Cardiology special*

## Echo report

Name: *[Signature]*

Date: 2/1/2021

|       |     |           |         |     |                    |
|-------|-----|-----------|---------|-----|--------------------|
| EF    | 62  | 55-75%    | PWT     | 0.9 | 0.6-1.1cm          |
| LVEDD | 4.2 | 3.5-5.8cm | AO R    | 3.8 | 2-3.7cm            |
| LVESD | 2.7 | 2-4cm     | LA      | 3.8 | 2-4cm              |
| FS    | 33  | 25-45%    | RV      |     | 0.7-2.3cm (supine) |
| SWT   | 0.9 | 0.6-1.1cm | AO CUSP |     | 1.5-2.6cm          |

LV dimensions *Normal*

LV wall thickness *Normal*

LV diastolic function *Normal*

SWMA *Normal*

LA and Ao root dimensions *Normal*

Mitral Valve *Normal*

Aortic valve *Normal*

Tricuspid valve *Normal*

Pulmonary valve *Normal*

Right side and systolic function *Normal*

Intracardiac masses and effusion *No masses or effusion*

*[Signature]*  
*[Signature]*  
*[Signature]*



Patient Name: *Hanan Shaaban Hassan.*

Date: 14/10/2021

### Tc-99m MDP bone scan.

Desired information : skeletal surveying to detect multiple lesions

#### Technique:

Delayed Anterior and posterior whole body scan 2 hours after I.V administration of 20 mCi <sup>99m</sup>Tc-MDP.

#### Delayed Static Phase:-

- Degenerative changes are noticed over dorsal and lumbar vertebrae for follow up.
- Otherwise the rest of surveyed skeleton is free from serious bone lesions.
- Both kidneys are seen.

#### Conclusion:

*No definite evidence of metastatic osseous deposits.*

*Follow up bone scan is advised.*

*Thanks for reference*

*Nuclear medicine Unit*

*Huda Fathy , M.D*



مبادرة رئيس الجمهورية  
لدعم صحة المرأة المصرية

السلطة الصحية  
مصر

MDT FORM

1-78678549

21/3/2022

|                   |            |                |              |
|-------------------|------------|----------------|--------------|
| Name              | فايزة محمد | MRN            | ٢٠٩٧٠٩٠٩١    |
| Age               | 51y        | ID             | ٩٧٠٩٧٠٩٧٠٩٧٠ |
| Menopausal status |            | Phone          | ٠١١٧٧٧٧٧٧٧٧  |
| DCP               |            | marital status |              |
| Comorbidity       |            | Family history |              |

Clinical Data

|                  |          |            |     |
|------------------|----------|------------|-----|
| Ex:              | INC 2 II | R          |     |
| EP:              | 7/1      | PR:        | 4/1 |
| HER2:            | +++      | Ki67:      |     |
| Sonamammography: |          |            |     |
| Mets Work up     | Free     |            |     |
|                  |          | CT 3M      |     |
|                  |          | → yPToNoMo | CPR |

Recommendations: A case of Rt Breast post BC+ST  
post RACT pathology: → pathological complete  
response SLNB → 0/5 pT0 pN0  
for post-operative re MDT

MDT OUTCOME & PLAN

Pr - Adj Trastuzumab SC. x 13 doses  
- PORT to Rt. Breast 2 SC  
- Adj hormonal th Arimidex x 54

|                  |         |           |           |
|------------------|---------|-----------|-----------|
| SIGNATURE        |         |           |           |
| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY |
|                  |         |           |           |

16 - 2 - 2022

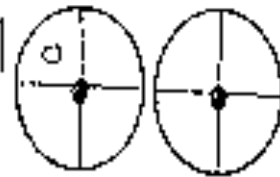


مبادرة رئيس الجمهورية  
لدعم صحة المرأة المصرية

البنيت المصرية  
صحة  
مصر

### MDT FORM

|              |                  |
|--------------|------------------|
| Name         | هناء محمد        |
| Age          | 51               |
| MRN          | 5.11.151057.5177 |
| ID           | 1111774440       |
| Co morbidity | C.51 / 4.5V      |



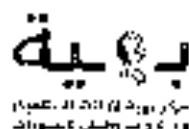
### Clinical Data

|                                                                                                   |              |
|---------------------------------------------------------------------------------------------------|--------------|
| Bx:                                                                                               | SM:          |
| IHC:<br>ER: 7/8<br>PR: 4/8<br>Her2: ++<br>KITV:                                                   | Mets Workup: |
| cT3 N1 → 4T1 N0 M0                                                                                |              |
| COMMENT:<br>received LAC → 12 wk Trial<br>+ 4 x Herceptin + Plavix<br>for CESM to a MR L Guidance |              |

### MDT OUTCOME & PLAN

|                                                                   |
|-------------------------------------------------------------------|
| for <del>BCS</del> BCS + SLN (≥ 4 LNs)<br>After wire localization |
|-------------------------------------------------------------------|

| SIGNATURE        |         |           |           |
|------------------|---------|-----------|-----------|
| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY |
|                  |         |           |           |



Med. Record No.: 2021158854  
Patient Name: HANAN SHABAN HASSAN AOSMAN  
Sex: Female DOR: 12/15/1973  
Nationality: EGYPTIAN  
Unit: External Clinic

### IMMUNOPATHOLOGY - Final Anatomical Diagnosis

#### Hormone receptors

Specimen Id 2107100020

Sample Type

Paraffin Block

Accession No IMM21-4204

Requesting Doctor

Abd ALJAF, Mohamed Gamil /

Collection Date 7/10/2021 8:54:08AM

Receive Date 7/10/2021 8:54:13AM

### Immunohistochemical Report

pathology number

504023

#### Clinical Data

RT, breast cancer

#### Gross Picture :

Referred block submitted for ER/PR immunostaining.

#### System Used :

Ventana BenchMark ULTRA autostainer

#### Antibody:

Ventana anti-PR(1E2) Rabbit monoclonal antibody, Ventana anti-ER(SP1) Rabbit monoclonal antibody.

#### Scoring system :

According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive. Allied assay: proportion of cells staining (0, no staining; 1, < 1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%; and 5, between 67%–100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8  
Comment: - There are much more limited data on the 2% to 3% of cancers that are low ER expression (most often defined as 1%-10% ER-positive cells by IHC) and the potential benefit from endocrine therapy. - PR positive in 1-20% of tumor cells is considered as low positive.

#### (Laterality) breast Cancer:

Right

#### Diagnosis :

ER: Positive, intermediate nuclear reaction in 70% of tumor cells (7/8)  
PR: Low Positive, intermediate nuclear reaction in 5% of tumor cells (4/8).

#### Signature

GHADA MOHAMED MD PHD.



11/93

وزارة الصحة  
وزارة الداخلية  
مستشفى الامام  
مركز اورام دار السلام  
قسم القلب والرئة  
قسم الموجات فوق الصوتية  
تاريخ اعداد الموجة  
رقم اعداد الموجة

وزارة الصحة والامكان  
امانة المراكز الطبية المتخصصة  
مركز اورام دار السلام

حبيب محمد

eport

Date: / / 2021

|       |     |           |         |        |                   |
|-------|-----|-----------|---------|--------|-------------------|
| EF    | 60  | 55-75%    | PWT     | 0.9    | 0.6-1.1cm         |
| LVEDD | 5.2 | 3.5-5.8cm | AO R    | 3.0    | 2-3.7cm           |
| LVEDD | 3.5 | 2-4cm     | LA      | 3.7    | 2-4cm             |
| FS    | 32  | 25-45%    | RV      | normal | 0.7-2.3cm(supine) |
| SWT   | 1.1 | 0.6-1.1cm | AO CUSP | normal | 1.5-2.6cm         |

LV dimensions normal

LV wall thickness normal

LV diastolic function normal

SWMA normal

LA and Aortic root dimensions normal

Mitral Valve thickened &amp; trivial MR

Aortic valve normal

Tricuspid valve normal &amp; trivial TR, normal R.V.P.

Pulmonary valve normal

Right side and systolic function normal

Intracardiac masses and effusion normal

D mohammed Anwar

Msc cardiology



وزارة الصحة والسكان  
أمانة المراكز الطبية المتخصصة  
مركز أورام دار السلام  
(هرمل سابقا)  
إدارة الجودة

## Doctor's Orders أوامر الطبيب

اسم المريض : .....  
الرقم الطبي : .....

### الأوامر

Chest: .....

Temp: .....

Oral mucositis: .....

Canula site infection: .....

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تماما ولسفقتن قوت كل عا كايه  
تالسا ح قوتها واد كل عا كايه  
وانا العنا قوتها واد كل عا كايه  
مولا كايه قوتها واد كل عا كايه  
نوقيع الطبيب : د/ رهنه مصطفى

التاريخ: ٢٠٢٠/٢/١٦  
الوقت:

### الأوامر

Chest: .....

Temp: .....

Oral mucositis: .....

Canula site infection: .....

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تماما ولسفقتن قوت كل عا كايه  
تالسا ح قوتها واد كل عا كايه  
وانا العنا قوتها واد كل عا كايه  
مولا كايه قوتها واد كل عا كايه  
نوقيع الطبيب : د/ رهنه مصطفى

التاريخ: ٢٠٢٠/٢/١٦  
الوقت:



**Doctor's Orders**    **أوامر الطبيب**

الاعمال

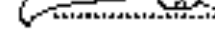
Canula site infection.....  
1/19/24 pet. Abs. No 2/24 → ho  
Abs. No 2/24 → ho

تولیع الطیبیہ :  
 محمد وسیم شاہ  
 صاحب  
 ۱۵

الأخوة

**Canula site infection.** 

24/11/2024 *[Signature]*

توقيع الطبيب : 

التاريخ: ١٣٣١ / ١١ / ١١  
الوقت: ١١:٥٢  
بدر



**Doctor's Orders** أوامر الطبيب

الامانة

6/2/2024      19      r      C. L. O'S      M.      p.      J.      J.

F.      r      C. L. O'S      J.      J.

J.      r      C. L. O'S      J.      J.

a      r      C. L. O'S      J.      J.

توقيع الطبيب :  
 م. السيد  
 ١٤٦٦

**Chest:** \_\_\_\_\_  
**Temp:** \_\_\_\_\_  
**Oral mucositis:** \_\_\_\_\_  
**Canula site infection:** \_\_\_\_\_

Canula site infection

دار الفجر  
الشارع  
الوقت:





وزارة الصحة والسكان  
أمانة المراكز الطبية المتخصصة  
مركز أورام دار السلام  
(هرمل سابقا)  
إدارة الجودة

Echo  
(23/11/2022) → 60 %

## Doctor's Orders أوامر الطبيب

اسم المريض: .....  
الرقم الطبي: ٢٠٢٤ / ٢٠٢٤ / ٢٠٢٤

### الأوامر

chest: S.H. →  
Temp: .....  
Oral mucositis: .....  
Canula site infection: .....

علاج المريض  
٢.٨٤٢ (١٠) ...  
١.٢٢٢ (١٠) ...  
٢.٨٤٢ (١٠) ...  
٢.٢٢٢ (١٠) ...

التاريخ:  
الوقت:

ph / Aya Fayed  
C16 (H) S-C  
23 / 11 / 2022

توقيع الطبيب:

### الأوامر

chest: .....  
Temp: .....  
Oral mucositis: .....  
Canula site infection: .....

علاج المريض  
٢.٨٤٢ (١٠) ...  
١.٢٢٢ (١٠) ...  
٢.٨٤٢ (١٠) ...  
٢.٢٢٢ (١٠) ...

التاريخ:  
الوقت:

ph / Aya Fayed  
C17 (H) S-C  
13 / 12 / 2022

توقيع الطبيب:





وزارة الصحة والسكان  
أمانة المراكز الطبية المتخصصة  
مركز أورام دار السلام  
(هرمل سابقاً)  
إدارة الجودة

## Doctor's Orders أوامر الطبيب

اسم المريض : حمادة شعيب  
الرقم الطبي :

### الأوامر

Chest: .....  
Temp: .....  
Oral mucositis: .....  
Canula site infection: .....

C12 Herceptin 500 mg SC

for Tamoxifen Tab 1x2 x3u

المريضة صرقت شهر واحد فقط

التاريخ: ٢٠٢٢ / ٨ / ٣١  
الوقت:

توقيع الطبيب : د. السامية السليمان

C12 Her  
aph Aya  
21/8

### الأوامر

Chest: .....  
Temp: .....  
Oral mucositis: .....  
Canula site infection: .....

المريضة صرقت شهر واحد فقط

for Tamoxifen Tab 1x2 x3u

المريضة صرقت شهر واحد فقط

for Tamoxifen Tab 1x2 x3u

المريضة صرقت شهر واحد فقط

التاريخ: ٢٠٢٢ / ٨ / ٣١  
الوقت:

C12 Her  
aph Aya  
21/8



وزارة الصحة والسكان  
أمانة المراكز الطبية المتخصصة  
مركز أورام دار السلام  
(هرمل سابقاً)  
إدارة الجودة

## Doctor's Orders أوامر الطبيب

اسم المريض : .....  
الرقم الطبي : .....

### الأوامر

Chest: .....

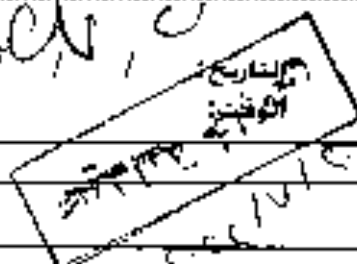
Temp: .....

Oral mucositis: .....

Canula site infection: .....

Herceptin 600 mg SC  
9/24/12  
on Ex. Free

CCN, C



C10 Herceptin  
cl.ph  
2017/2012

د. كريمة

توقيع الطبيب :

### الأوامر

Chest: .....

Temp: .....

Oral mucositis: .....

Canula site infection: .....

EF = 61% 10/8/12

C10 Herceptin 600 mg SC

C10 H  
aphaga  
10/16

التاريخ: 10/1/12  
الوقت:

د. كريمة

توقيع الطبيب :



وزارة الصحة والسكان  
أمانة المراكز الطبية المتخصصة  
مركز أورام دار السلام  
(هرمل سابقا)  
إدارة الجودة

## Doctor's Orders أوامر الطبيب

اسم المريض : .....  
الرقم الطبي : .....

### الأوامر

Chest: .....

Temp: .....

Oral mucositis: 7/6/2022

Canula site infection: .....

Handwritten notes and signatures in Arabic, including a large signature and a stamp. The text includes "أوامر الطبيب" (Doctor's Orders) and "توقيع الطبيب" (Signature of the Doctor). The date "7/6/2022" is written at the bottom.

### الأوامر

Chest: .....

Temp: 38.4°C

Oral mucositis: .....

Canula site infection: .....

Handwritten notes and signatures in Arabic, including a large signature and a stamp. The text includes "أوامر الطبيب" (Doctor's Orders) and "توقيع الطبيب" (Signature of the Doctor). The date "23/6/2022" is written at the bottom.



وزارة الصحة والسكان  
أمانة المراكز الطبية المتخصصة  
مركز أورام دار السلام  
( هومل سابقا )  
إدارة الجودة

## Doctor's Orders أوامر الطبيب

EF627.

26.14.2022

اسم المريض :

الرقم الطي: ١

الأوامر

```
chest:
```

Terima kasih

Oral mucositis: -

Canula site infection, 51, 52, 53, 54

7. \_\_\_\_\_ - C

### جہاں سے آؤں

CLX 71

[illegible]

CH

C. D. Ayala

26141-2612

6-6-1967

مرامعه کامل

توقيع الطبيب : \_\_\_\_\_

## التاريخ:

الوقت:

CG 611

الأوامر  $\pi$  الحاصلة من

chest:.....

Temp: \_\_\_\_\_

Oral impingement: 2/4

Canula site infection.....

١٥٦، ١٥٧، ١٥٨

عبد الرحمن بن عبد الله

Handwritten:  $\frac{1}{2} \text{ mole } C_2H_4 + \frac{1}{2} \text{ mole } H_2$

2. Chapman

القنطرة

4-121522

**توقيع الطبيب :**

## نتائج:

وقت:



وزارة الصحة والسكان  
أمانة المراكز الطبية المتخصصة  
مركز أورام دار السلام  
(هرمل سابقاً)  
إدارة الجودة

## Doctor's Orders أوامر الطبيب

اسم المريض: .....  
الرقم الطبي: .....

51 سنة

### الأوامر

Chest: .....

Temp: .....

Oral mucositis: .....

Canula site infection: .....

ER 7/8 - PR 4/8

K67

At Breast Ca

HER ++

→ PT. No. No → PCR achieved

For ad: trastuzumab 600 mg sc

X13 doses

توقيع الطبيب:

التاريخ:  
الوقت:

أحمد حسين محمد حسن

### الأوامر

Chest: .....

Temp: .....

Oral mucositis: .....

Canula site infection: .....

ophth. B

8/5/4/2022

X1X1  
X1X1

سجل  
دلتا

5/2/0

توقيع الطبيب:

التاريخ:  
الوقت:



وزارة الصحة والسكان  
أمانة المراكز الطبية المتخصصة  
مركز أورام دار السلام  
(هرمل سابقاً)  
إدارة الجودة

## Doctor's Orders أوامر الطبيب

اسم المريض : .....  
الرقم الطبي : .....

### الأوامر

chest: .....  
Temp: .....  
Oral mucositis: .....  
Canula site infection: ..... 514 e anal abscess (chronic)?

for your kind care

1/1/9 تاريخ الطبيب

توقيع الطبيب : .....

1 / 1 التاريخ:  
الوقت:

### الأوامر

chest: .....  
Temp: .....  
Oral mucositis: .....  
Canula site infection: ..... patient complains of peripheral neuropathy

RI Neurovit tab 1x1x3

RI Gafin 300mg 1x2x3

1/1/9 التاريخ:  
الوقت:

توقيع الطبيب : .....



مركز اخصية المتخصصة  
مستشفى اورام دار السلام ( هرمس )  
الصيدلة الاكلينيكية

### TC<sub>4</sub>HP (with weekly Paclitaxel ) cycle (.....)

|                                                                                                                                                                                                                                                                                                                    |                                                                                                                                                                                                      |               |                                     |
|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------|-------------------------------------|
| التاريخ: 2022/...../.....                                                                                                                                                                                                                                                                                          |                                                                                                                                                                                                      |               |                                     |
| اسم المريض: ..... رقم الملف: .....                                                                                                                                                                                                                                                                                 |                                                                                                                                                                                                      |               |                                     |
| Weight: ..... kg                                                                                                                                                                                                                                                                                                   | Height: ..... cm                                                                                                                                                                                     | BSA: ..... M2 | Age: ..... years                    |
| CBC:                                                                                                                                                                                                                                                                                                               | WBC: .....                                                                                                                                                                                           | ANC: .....    | Hgb: ..... PLT: .....               |
| Functions Tests:                                                                                                                                                                                                                                                                                                   | Sr.Creat: .....                                                                                                                                                                                      | Urea: .....   | CrCL: .....                         |
|                                                                                                                                                                                                                                                                                                                    | ALT: .....                                                                                                                                                                                           | AST: .....    | T.Bil: ..... D.Bil: .....           |
| Echo:                                                                                                                                                                                                                                                                                                              | Date: .....                                                                                                                                                                                          | EF: ..... %   |                                     |
| Dose Modification: ..... %                                                                                                                                                                                                                                                                                         |                                                                                                                                                                                                      |               |                                     |
| Reason: <input type="checkbox"/> Hematological <input type="checkbox"/> Non hematological: .....                                                                                                                                                                                                                   |                                                                                                                                                                                                      |               |                                     |
| امضاء التمريض:                                                                                                                                                                                                                                                                                                     | بروتوكول العلاج                                                                                                                                                                                      |               | المسرف                              |
| الاسبوع الاول:                                                                                                                                                                                                                                                                                                     | جرانيسرون (0.01 مجم/كجم) ..... مجم +<br>ديكساميثازون 20 مجم + لتودين 20 مجم + امبول افيل)<br>في 50 مل ح 5% على مدى ربع ساعة قبل العلاج<br>الكماوي.<br>(Maximum dose of Granisetron = 1mg)            |               | الاسبوع 1:<br>بتاريخ 19/...../..... |
| الاسبوع الثاني:                                                                                                                                                                                                                                                                                                    | بالترياكسيل (80 مجم/2) 1.5.2 مجم في 250 مل ح<br>5% على مدى ساعة.                                                                                                                                     |               | بتاريخ 19/...../.....               |
| الاسبوع الثالث:                                                                                                                                                                                                                                                                                                    | فازيفين (AUC 3) ..... مجم على مدى ساعة                                                                                                                                                               |               | الاسبوع 1:<br>بتاريخ 19/...../..... |
|                                                                                                                                                                                                                                                                                                                    | تراستوزوماب 600 مجم تحت الجلد لمدة 5 دقائق في<br>الجلد<br>بروتوزوماب (7.7) مجم في 250 مل ملح على مدى 1/2<br>ساعة (على مدى ساعة في الجرعة الاولى)<br>Loading Dose: 840 mg<br>Maintenance Dose: 420 mg |               | بتاريخ 19/...../.....               |
| كورولوك قرص 40 مجم على تريق يوميا<br>ديكساميثازون 8 مجم (امبول او قرص) مرة يوميا اليوم الثاني و الثالث.<br><input checked="" type="checkbox"/> جرانيسرون قرص 1 مجم كل 12 ساعة اليوم الثاني و الثالث.<br><input type="checkbox"/> اوندالينز 8 مجم قرص كل 12 ساعة اليوم الثاني و الثالث.<br>فلاجيل 500 ..... 1 3 2 1 |                                                                                                                                                                                                      |               |                                     |
| Physician Signature:                                                                                                                                                                                                                                                                                               | Clinical pharmacy:                                                                                                                                                                                   |               | Dispensing pharmacy:                |
| .....                                                                                                                                                                                                                                                                                                              | W10 >> Taxol /C <sub>4</sub> HP 19/11/2022<br>W11 >> Taxol 28/11/2022<br>W12 >> Taxol Dose B 2/2<br>trastuzumab + pertuzumab ><br>Date .....                                                         |               | .....<br>.....<br>.....             |



Ministry OF Health  
Dar El-Salam Cancer Hospital  
Clinical Pharmacy Team

ph: Aya Fayed  
12/11/2021  
W7 (taxol)

توفد الجيرة القائمة  
بتحاليل جديدة

ADW  
12/11/2021  
12/11/2021

# TCHP (with weekly Taxol) cycle (.....)

التاريخ: 2021 / ..... / .....  
المرضى: .....  
اسم المريض: .....  
Weight: 87 kg Height: 155 cm BSA: 1.9 M2 Age: ..... years  
CBC: WBC: ..... ANC: ..... Hgb: ..... PLT: .....  
Functions Tests: Sr.Creat: ..... Urea: ..... CrCL: .....  
ALT: ..... AST: ..... T.Bil: ..... D.Bil: .....  
Echo: Date: ..... EF: ..... %

## Dose Modification:

Reason: ☐ Hematological ☐ Non hematological:

| امضاء التمريض   | بروتوكول العلاج                                                                                                                                                                        | انصرف                                |
|-----------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------|
| الاسبوع الاول:  | (جر انقريب (D.01 مج/كجم) ..... مج +<br>نيكسيميثازون 20 مج + ستونين 20 مج + امبول<br>في 50 مل ج 5% على مدى ربع ساعة قبل العلاج<br>الكيمائي.<br>(Maximum dose of Granisetron = 1mg)      | الاسبوع 1<br>بتاريخ 12/11/2021       |
| الاسبوع الثاني: | تفصول (80 مج/2) 15.2 مج في 250 مل ج 5%<br>على مدى ساعة                                                                                                                                 | الاسبوع 2<br>بتاريخ 13/11/2021       |
| الاسبوع الثالث: | تفصول (80 مج/2) 15.2 مج في 250 مل ج 5%<br>على مدى ساعة                                                                                                                                 | الاسبوع 3<br>بتاريخ 14/11/2021       |
|                 | هرميتين 600 مج تحت الجلد لمدة 5 دقائق في الفخذ<br>بريفينا 250 مج في 250 مل منع على مدى 1/2 ساعة<br>(على مدى ساعة في الجرعة الاولى)<br>Loading Dose: 840 mg<br>Maintenance Dose: 420 mg | يعطى بتاريخ<br>..... / ..... / ..... |

EF 88%  
23/11/2021

العلاج التدايمي:  
هيدروكورتيزون 40 مج على طريق يوميا  
نيكسيميثازون 8 مج (امبول او قرص) مرة يوميا اليوم الثاني والثالث  
جر انقريب قرص 1 مج كل 12 ساعة اليوم الثاني والثالث  
لووندايفز 8 مج قرص كل 12 ساعة اليوم الثاني والثالث

Physician Signature: .....  
Clinical pharmacy: .....  
Dispensing pharmacy: .....  
W7 >> 12/11/22  
Herceptin + perjeta > Date: 12/11/22

12/11/22



وزارة الصحة والسكان  
أمانة المراكز الطبية المتخصصة  
مركز أورام دار السلام  
(هرمل سابقاً)  
إدارة الجودة

## Doctor's Orders أوامر الطبيب

اسم المريض : .....

الرقم الطبي : .....

### الأوامر

Chest: .....

Temp: .....

Oral mucositis: .....

Canula site infection: .....

Handwritten notes in Arabic, including "Handwritten notes" and "Handwritten notes".

التاريخ:  
الوقت:

توقيع الطبيب:

### الأوامر

Chest: .....

Temp: .....

Oral mucositis: .....

Canula site infection: .....

Handwritten notes in Arabic, including "Handwritten notes" and "Handwritten notes".

التاريخ:  
الوقت:

توقيع الطبيب:

Handwritten signature and date: 11/05/2020